

Alcohol Use Disorder: Patient Information Leaflet

A problematic pattern of alcohol use leading to significant distress or impairment, ranging from mild to severe based on the number of features present.

What is Alcohol Use Disorder?

Alcohol Use Disorder (AUD) is a chronic, relapsing pattern of alcohol use characterized by an inability to stop or control drinking despite adverse social, occupational, or health consequences. DSM-5 combined the previously separate categories of 'alcohol abuse' and 'alcohol dependence' into a single disorder existing on a spectrum of severity — mild, moderate, or severe — based on the number of diagnostic criteria met.

Common symptoms

- Drinking more or for longer than intended
- Persistent desire or unsuccessful efforts to cut down
- Significant time spent obtaining, using, or recovering from alcohol
- Strong cravings
- Failure to fulfill major role obligations at work, school, or home
- Continued use despite social or relationship problems caused by drinking
- Giving up important activities in favor of drinking
- Recurrent use in physically hazardous situations

What causes it

AUD has a strong genetic component, with heritability estimated at 50–60% — among the more heritable substance use disorders. Alcohol increases dopamine release in

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withdrawal. Psychosocial factors — family or peer alcohol use, trauma, chronic stress, cultural drinking norms, and co-occurring psychiatric conditions such as anxiety, depression, or PTSD — commonly contribute to and complicate the disorder.

Treatment options

Medically supervised detoxification is recommended for those with significant physical dependence, given seizure and delirium tremens risk, followed by psychosocial treatment and often medication to support abstinence and reduce relapse risk.

Treatment is tailored to severity and the individual's goals.

Self-help tips

- Identify and avoid personal drinking triggers
- Build a sober support network
- Engage in alternative rewarding activities to replace drinking
- Address co-occurring sleep, anxiety, or depression symptoms, which can otherwise drive relapse
- Have a clear relapse plan recognizing early warning signs and a specific response

Prognosis

AUD is a chronic, relapsing condition similar to other chronic diseases, but meaningful, lasting recovery is achievable for many people with treatment. Relapse is common and should be understood as part of the recovery process for many, rather than a treatment failure. Untreated, AUD is associated with significant physical health complications and markedly increased mortality risk.

When to seek urgent help

Seek emergency care for signs of alcohol withdrawal (tremor, sweating, agitation, hallucinations) in someone attempting to stop after heavy use, which can progress to seizures or delirium tremens; for severe intoxication with risk of

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unconscious); or for suicidal ideation, which has an elevated association with AUD.

This material is for patient and family education only and does not replace individualized medical advice. For a personal diagnosis or treatment plan, consult Dr. Kushal Kharel or a qualified mental health professional.

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